



Acceptance Letter

Paper ID	Paper Title	Author(s)
ICSCDS-018	ElderMeds: A Multimodal Voice Vision Based Intelligent Assistant for Medication Adherence	Dewmini Christine, Sanuji Silva, Sandali Perera, Thyaga Alwis, Hansi De Silva, Samantha Thelijjagoda,

Decision: Acceptance with Major Revision

Herewith, the conference committee of the **4th International Conference of Sustainable Computing and Data Communication Systems [ICSCDS 2026]** is pleased to inform you that the above-mentioned research article has been **accepted** for presentation and publication in ICSCDS Conference Proceedings. ICSCDS-2026 will be held during 15-17, December 2026 at Shree Venkateshwara Hi-tech Engineering College, India.

We congratulate you on being successfully selected for the presentation and publication of your research work in our esteemed conference.

Best Regards,

Dr. K. M. Arunraja
Conference Chair - ICSCDS-2026



Proceedings by



REVIEW COMMENTS

Review Comments:1-

1. Clarify how outputs from individual modules are persisted and subsequently retrieved through the shared data layer.
2. Explain the fallback relationship between MediaPipe landmarks and OpenCV in the hand-to-mouth motion stage.
3. Use consistent terminology for “risk-awareness”, “disease-risk” and “risk prediction” throughout the manuscript.
4. Authors should specify how the Unified Conversational Dashboard connects retrieved records with grounded response generation.
5. Define the P1-P16 rule catalogue before referring to its prioritization in the methodology.

Review Comments:2

1. The reported pill-counting performance should be validated on a larger and more diverse dataset beyond 860 images with an independent external test set.
2. The medication-safety controller results are based on only ten controlled scenarios, which is insufficient to establish clinical reliability or generalizability.
3. The three-class safety calibration uses proxy labels rather than clinician-adjudicated ground truth, so the reported accuracy and F1-score cannot confirm real clinical safety.
4. The emotion-classification evaluation uses only 140 domain responses, and class distribution, independent validation, and confidence intervals should be reported to verify the macro-F1 result
5. The disease-risk results require stronger validation because stroke shows low precision and hypertension performs near random, limiting the reliability of the claimed risk-awareness capability.
6. References 14-18 are not cited in the article. Please check and update.
7. Tables 1,3-7 are not cited in the article. Please check and update.
8. Minor language editing is required to eliminate grammatical errors.